Wound CTP (Skin Substitute Grafting) Note

Patient Information

Field	Entry
Patient Name	John Doe
MRN	145789
DOB	02/14/1958
Date of Service	09/23/2025
Provider	Dr. Sarah Williams, DPM
Location	Executive Hawks Wound Care Center

Reason for Visit / Chief Complaint

- **Application of cellular/tissue-based product (CTP) for multiple chronic non-healing diabetic foot ulcers and venous ulcer.**

History of Present Illness (HPI)

- **Onset of wounds:** July 2025
- **Duration:** ~12 weeks
- **Standard care attempted (≥4 weeks):**
  - Sharp debridement
  - Moist wound dressings (alginate, hydrocolloid, foam)
  - Compression (for venous ulcer)
  - Offloading shoe insert (for plantar wounds)
  - Glycemic control optimization (HbA1c 7.9 → 7.2)
- **Compliance:** Fair (occasional missed appointments, inconsistent offloading)
- **Current wound status:** Chronic, non-healing despite SOC, moderate drainage, no systemic infection

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Review of Systems

Symptom	Present (Y/N)
Fever/Chills	N

Symptom	Present (Y/N)
Malaise	N
Pain at wound sites	Y (mild, intermittent 3/10)
Drainage/Odor	Y (moderate serosanguinous, no foul odor)
Other systemic complaints	N

Physical Exam

- **General:** Well-appearing, in no acute distress
- **Vitals:** BP: 132/78 | HR: 84 | Temp: 98.1°F | SpO_2: 97%
- **Extremities:** Mild edema RLE, intact pedal pulses, diminished plantar sensation, limited mobility
- **Vascular Assessment:** ABI Right: 1.0, Left: 1.1

Complete At Risk Foot Exam

- **Class Findings:**

1. **Class B: (1**) decreased hair growth, nail thickening, and discoloration
2. **Class C: (2**) edema and cold feet

The patient meets Class Findings Criteria by having at least 1 Class B Finding and 2 Class C Findings.

- **Gait:** standard walker. limited household ambulator.

- **Right Shoe Size:** 12.0

1. **Width:** Wide
2. **Measuring Device:** Brannock Device

- **Left Shoe Size:** 12.0

1. **Width:** Wide
2. **Measuring Device:** Brannock Device

- **At Risk Foot Exam**
- **Peripheral Pulses**:

1. **Right Dorsalis Pedis**: +1 faint but detectable dorsalis pedis pulse
2. **Right Posterior Tibial**: +1 faint but detectable posterior tibial pulse
3. **Right Capillary Refill**: Normal CFT
4. **Right Vascular Skin Trophic Changes**: absent hair growth, decreased temperature gradient, pigmentation changes, and cool

5. **Right Edema**: Non pitting
6. **Right Venous Exam**: telangiectasia
7. **Sensation**: Right LE: Normal peripheral nerve sensation

- **Peripheral Pulses:**

1. **Left Dorsalis Pedis:** +1 faint but detectable dorsalis pedis pulse
2. **Left Posterior Tibial:** +1 faint but detectable posterior tibial pulse
3. **Left Capillary Refill**: Normal CFT
4. **Left Vascular Skin Trophic Changes:** absent hair growth, decreased temperature gradient, pigmentation changes, and cool
5. **Left Edema:** Non pitting
6. **Left Venous Exam**: telangiectasia
7. **Sensation Left LE:** Normal peripheral nerve sensation

- **Skin:**

Right Foot and Ankle: ulcer Wound bed granular less fat layer within the wound bed than previous. Does not probe deep. No fluctuance or purulence noted. No

other local signs of infection.

- **Toenails:**

Right: Normal

- **Inspection:**

1. Right Foot and Ankle: Charcot arthropathy
2. Appearance: well developed and nourished
3. Orientation: Alert and oriented to person, place, time.

- **Additional Exam Findings:**

1. Wound Width Pre-treatment in cm: 1.9
2. Wound Depth Pre-treatment in cm: .3
3. Wound Length Pre-treatment in cm: 2.9

- **Skin:**

Left Foot and Ankle: there is normal texture, temperature, turgor and color of the skin

Toenails:

Left: Normal

- **Inspection:**

Left Foot and Ankle: Charcot Arthropathy

Wound Examination (All Wounds)

Wound #	Location	Etiology	Stage	Length (cm)	Width (cm)	Depth (cm)	SA (cm^2)	Volume (cm^3)
1	Right plantar foot (1st met head)	Diabetic Foot Ulcer (DFU)	Wagner II	2.8	2.2	0.4	6.16	2.46
2	Right lateral malleolus	Venous stasis ulcer	Stage III	3.5	2.0	0.3	7.0	2.1
3	Left plantar forefoot	DFU	Wagner I	1.5	1.2	0.2	1.8	0.36
4	Left heel	Pressure ulcer	Stage II	2.0	1.8	0.2	3.6	0.72

Wound Bed Details (all wounds):

- **Wound 1:** 70% granulation, 25% slough, 5% eschar; moderate serosanguinous drainage; no odor
- **Wound 2:** 60% granulation, 35% slough, 5% eschar; moderate drainage; mild periwound erythema, no cellulitis
- **Wound 3:** 80% granulation, 20% slough; scant drainage; clean periwound
- **Wound 4:** 75% granulation, 25% slough; mild drainage; no odor

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Assessment

- **Wound types:**
  - Wound 1 – Chronic non-healing diabetic ulcer (Wagner II, plantar)
  - Wound 2 – Chronic venous stasis ulcer, lateral ankle
  - Wound 3 – Chronic diabetic foot ulcer, forefoot (Wagner I)
  - Wound 4 – Stage II pressure ulcer, left heel
- **Contributing factors:** Diabetes, neuropathy, venous stasis, pressure, compliance issues
- **Risks if untreated:** Local infection, osteomyelitis, hospitalization, amputation
- **Medical necessity:** Multiple chronic wounds refractory to SOC ≥4 weeks; advanced therapy (CTP) required to prevent deterioration

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Procedure Performed

- **Indication:** Multiple chronic, non-healing ulcers unresponsive to ≥4 weeks of SOC
- **Consent:** Obtained, risks & benefits reviewed
- **Preparation:** Wounds cleansed with sterile saline; sharp debridement of necrotic/slough tissue performed:
  - Wound 1: ~25% slough removed
  - Wound 2: ~30% slough removed
  - Wound 3: ~10% slough removed
  - Wound 4: ~15% slough removed
- **Hemostasis:** Achieved with pressure
- **CTP Applied:**
  - Product: Apligraf® (44 cm^2 sheet), Lot #A12547, Exp: 05/2026
  - Units Applied:
    - Wound 1: 6.16 cm^2
    - Wound 2: 7.0 cm^2
    - Wound 3: 1.8 cm^2
    - Wound 4: 3.6 cm^2
    - **Total used:** 18.56 cm^2
    - **Wastage documented:** 25.44 cm^2 discarded
- **Application details:** Each graft trimmed to wound size, applied directly to wound beds, secured with non-adherent dressing, foam bolster, and compression (ankle ulcer) / offloading (plantar & heel ulcers)
- **Secondary Dressing:** Foam + protective gauze wrap
- **Tolerance:** Patient tolerated well with local lidocaine
- **Estimated blood loss:** Minimal (<5 mL total)
- **Complications:** None

Medical Necessity

- Meets LCD criteria: SOC ≥4 weeks, infection excluded, vascular supply adequate, comorbidities managed
- Prevents deterioration, infection, or limb loss
- Wound bed prepared with viable granulation tissue

Plan

- **Cleansing:** Sterile saline with each dressing change
- **Debridement:** Weekly sharp debridement PRN(10x10cm)
- **Dressings:** Non-adherent over grafts, foam/gauze secondary, change weekly unless saturated
- **Compression:** Continue multilayer wrap on right ankle ulcer

- **Offloading:** Diabetic shoe with custom insole + heel cushion for left heel ulcer
- **Nutritional support:** High-protein diet, vitamin C & zinc encouraged
- **Pain management:** Acetaminophen PRN
- **Follow-up:** Weekly wound clinic visit; consider repeat grafting in 7–14 days if necessary
- **Escalation plan:** If inadequate progress, consider NPWT or HBOT

Post-Procedure Care

- **Dressing instructions:** Keep dressings intact for 7 days; reinforce outer layer if needed
- **Monitoring:** Report increased pain, erythema, drainage, or graft displacement
- **Comorbidity optimization:** Continue diabetes and vascular management
- **Follow-up:** 09/30/2025 – wound reassessment and possible 2nd graft

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Diagnosis

Diagnosis	Code	Comments
Diabetic ulcer of right foot, Wagner II	E11.621, L97.412	Chronic, non-healing
Venous ulcer, right ankle, stage III	I83.019, L97.313	Chronic, non-healing
Diabetic ulcer of left foot, Wagner I	E11.621, L97.422	Chronic, non-healing
Pressure ulcer, left heel, stage II	L89.612	Chronic, non-healing

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Lab Tests

- CBC (rule out occult infection)
- HbA1c (most recent 7.2 on 08/2025)

Follow-Up

Type	Date	Notes
Office	09/30/2025	Reassess all 4 wounds, consider additional grafts

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Notes:

Patient educated on pressure injury prevention, including frequent repositioning, offloading devices, skin care, and nutrition. Maintain offloading, moisture control, and pressure reduction strategies

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Referring Provider: Dr. Alan Shaw, MD
Primary Care Provider: Dr. Karen Lee, MD
Primary Provider of Record (Billing): Dr. Sarah Williams, DPM

✅ **Medicare Compliance:**

- 4 wounds documented with measurements, % tissue, exudate, and SOC
- CTP product details (lot #, size, expiration, wastage) documented
- ≥4 weeks SOC confirmed
- Skilled intervention documented
- Medical necessity justified
- Follow-up plan documented